Name	: Mrs. Supriya Mahatme		
PID No	: DRM54682	Register On	: 01/06/2024 8:59 AM
Visit ID	: 16232409834	Collection On	: 01/06/2024 9:44 AM
Age / Sex	: 35 Year(s) / Female	Printed On	: 03/06/2024 1:47 PM
Type	: OP	Report On	: 01/06/2024 7:34 PM
Ref. Dr	: DR ADEEB MOHAMMED		

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Investigation	Observed Value	Unit	Biological Reference Interval
HAEMATOLOGY			
Complete Haemogram			
Haemoglobin (EDTA Blood/ Spectrophotometry)	**11.1**	g/dL	12.5 - 16.0
INTERPRETATION: Haemoglobin values vary in Men, Women & Children. Low haemoglobin values may be due to nutritional deficiency, blood loss, renal failure etc. Higher values are often due to dehydration, smoking , high altitudes , hypoxia etc.			
Remark: Kindly correlate clinically			
PCV (Packed Cell Volume) / Haematocrit (EDTA Blood/Derived)	**35.0**	%	37 - 47
RBC Count (EDTA Blood/Automated Blood cell Counter)	**3.79**	mill/cu.mm	4.2 - 5.4
MCV (Mean Corpuscular Volume) (EDTA Blood/Derived from Impedance)	92.0	fL	78 - 100
MCH (Mean Corpuscular Haemoglobin) (EDTA Blood/Derived)	29.3	pg	27 - 32
MCHC (Mean Corpuscular Haemoglobin concentration) (EDTA Blood/Derived)	**31.7**	g/dL	32 - 36
RDW-CV(Derived)	12.8	%	11.5 - 16.0
RDW-SD(Derived)	41.22	fL	39 - 46
Total WBC Count (TC) (EDTA Blood/Derived from Impedance)	5340	cells/cu.mm	4000 - 11000
Neutrophils (Blood/Impedance Variation & Flow Cytometry)	58	%	40 - 75
Lymphocytes (Blood/Impedance Variation & Flow Cytometry)	34	%	20 - 45
Eosinophils (Blood/Impedance Variation & Flow Cytometry)	03	%	01 - 06
Monocytes (Blood/Impedance Variation & Flow Cytometry)	04	%	01 - 10
Basophils (Blood/Impedance Variation & Flow Cytometry)	01	%	00 - 02
Absolute Neutrophil count (EDTA Blood/ Impedance Variation & Flow Cytometry)	3.10	10^3 / µl	1.5 - 6.6
Absolute Lymphocyte Count (EDTA Blood/ Impedance Variation & Flow Cytometry)	1.82	10^3 / µl	1.5 - 3.5
Absolute Eosinophil Count (AEC) (EDTA Blood/Impedance Variation & Flow Cytometry)	0.16	10^3 / µl	0.04 - 0.44
Absolute Monocyte Count (EDTA Blood/ Impedance Variation & Flow Cytometry)	0.21	10^3 / µl	< 1.0
Absolute Basophil count (EDTA Blood/ Impedance Variation & Flow Cytometry)	0.05	10^3 / µl	< 0.2

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Consultant Pathologist
KMC No: 86542

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Investigation	Observed Value	Unit	Biological Reference Interval
Platelet Count (EDTA Blood/Derived from Impedance)	308	10^3 / µl	150 - 450
MPV (Blood/Derived)	9.2	fL	8.0 - 13.3
PCT	0.28	%	0.18 - 0.28
ESR (Erythrocyte Sedimentation Rate) (Citrated Blood/Automated ESR analyser)	16	mm/hr	< 20
RDW (Blood)	14	%	12 - 15

BIOCHEMISTRY

Glucose Fasting (FBS) (Plasma - F/GOD-POD)	93	mg/dL	Normal: < 100 Pre Diabetic: 100 - 125 Diabetic: >= 126

INTERPRETATION: Factors such as type, quantity and time of food intake, Physical activity, Psychological stress, and drugs can influence blood glucose level.

Glucose Postprandial (PPBS) (Plasma - PP/ GOD - POD)	85	mg/dL	70 - 140

INTERPRETATION:
Factors such as type, quantity and time of food intake, Physical activity, Psychological stress, and drugs can influence blood glucose level. Fasting blood glucose level may be higher than Postprandial glucose, because of physiological surge in Postprandial Insulin secretion, Insulin resistance, Exercise or Stress, Dawn Phenomenon, Somogyi Phenomenon, Anti- diabetic medication during treatment for Diabetes.

Blood Urea Nitrogen (BUN) (Serum/Urease UV / derived)	11.2	mg/dL	7.0 - 21
Creatinine (Serum/Jaffe Kinetic)	1.0	mg/dL	0.6 - 1.1

INTERPRETATION: Elevated Creatinine values are encountered in increased muscle mass, severe dehydration, Pre-eclampsia, increased ingestion of cooked meat, consuming Protein/ Creatine supplements, Diabetic Ketoacidosis, prolonged fasting, renal dysfunction and drugs such as cefoxitin ,cefazolin, ACE inhibitors ,angiotensin II receptor antagonists,N-acetylcyteine , chemotherapeutic agent such as flucytosine etc.

Uric Acid (Serum/Uricase/Peroxidase)	**2.4**	mg/dL	2.6 - 6.0

Liver Function Test

Bilirubin(Total) (Serum/Diazotized Sulfanilic Acid)	0.3	mg/dL	0.1 - 1.2
Bilirubin(Direct) (Serum/Diazotized Sulfanilic Acid)	0.1	mg/dL	0.0 - 0.3
Bilirubin(Indirect) (Serum/Derived)	0.20	mg/dL	0.1 - 1.0
Total Protein (Serum/Biuret)	6.7	gm/dl	6.0 - 8.0
Albumin (Serum/Bromocresol green)	4.4	gm/dl	3.5 - 5.2
Globulin (Serum/Derived)	2.30	gm/dL	2.3 - 3.6
A : G Ratio (Serum/Derived)	1.91		1.1 - 2.2

INTERPRETATION: Remark : Electrophoresis is the preferred method

SGOT/AST (Aspartate Aminotransferase) (Serum/IFCC / Kinetic)	33	U/L	5 - 40

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Investigation	Observed Value	Unit	Biological Reference Interval
SGPT/ALT (Alanine Aminotransferase) (Serum/IFCC / Kinetic)	24	U/L	5 - 41
Alkaline Phosphatase (SAP) (Serum/PNPP / Kinetic)	84	U/L	42 - 98
GGT(Gamma Glutamyl Transpeptidase) (Serum/IFCC / Kinetic)	25	U/L	< 38
Lipid Profile			
Cholesterol Total (Serum/Oxidase / Peroxidase method)	168	mg/dL	Optimal: < 200 Borderline: 200 - 239 High Risk: >= 240
Triglycerides (Serum/Glycerol phosphate oxidase / peroxidase)	**196**	mg/dL	Optimal: < 150 Borderline: 150 - 199 High: 200 - 499 Very High: >= 500

INTERPRETATION: The reference ranges are based on fasting condition. Triglyceride levels change drastically in response to food, increasing as much as 5 to 10 times the fasting levels, just a few hours after eating. Fasting triglyceride levels show considerable diurnal variation too. There is evidence recommending triglycerides estimation in non-fasting condition for evaluating the risk of heart disease and screening for metabolic syndrome, as non-fasting sample is more representative of the "usual" circulating level of triglycerides during most part of the day.

HDL Cholesterol (Serum/Immunoinhibition)	**41**	mg/dL	Optimal(Negative Risk Factor): >= 60 Borderline: 50 - 59 High Risk: < 50
LDL Cholesterol (Serum/Calculated)	87.8	mg/dL	Optimal: < 100 Above Optimal: 100 - 129 Borderline: 130 - 159 High: 160 - 189 Very High: >= 190
VLDL Cholesterol (Serum/Calculated)	**39.2**	mg/dL	< 30
Non HDL Cholesterol (Serum/Calculated)	127.0	mg/dL	Optimal: < 130 Above Optimal: 130 - 159 Borderline High: 160 - 189 High: 190 - 219 Very High: >= 220

INTERPRETATION: 1.Non-HDL Cholesterol is now proven to be a better cardiovascular risk marker than LDL Cholesterol. 2.It is the sum of all potentially atherogenic proteins including LDL, IDL, VLDL and chylomicrons and it is the "new bad cholesterol" and is a co-primary target for cholesterol lowering therapy.

Total Cholesterol/HDL Cholesterol Ratio (Serum/Calculated)	4.1	Optimal: < 3.3 Low Risk: 3.4 - 4.4 Average Risk: 4.5 - 7.1 Moderate Risk: 7.2 - 11.0 High Risk: > 11.0

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Investigation	Observed Value	Unit	Biological Reference Interval
Triglyceride/HDL Cholesterol Ratio (TG/HDL) (Serum/Calculated)	4.8		Optimal: < 2.5 Mild to moderate risk: 2.5 - 5.0 High Risk: > 5.0
LDL/HDL Cholesterol Ratio (Serum/ Calculated)	2.1		Optimal: 0.5 - 3.0 Borderline: 3.1 - 6.0 High Risk: > 6.0
Glycosylated Haemoglobin (HbA1c)			
HbA1C (Whole Blood/HPLC)	05.2	%	Normal: 4.5 - 5.6 Prediabetes: 5.7 - 6.4 Diabetic: >= 6.5

INTERPRETATION: If Diabetes - Good control : 6.1 - 7.0 % , Fair control : 7.1 - 8.0 % , Poor control >= 8.1 %

Estimated Average Glucose (Whole Blood)	102.54	mg/dl	

INTERPRETATION: Comments
HbA1c provides an index of Average Blood Glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.
Conditions that prolong RBC life span like Iron deficiency anemia, Vitamin B12 & Folate deficiency, hypertriglyceridemia,hyperbilirubinemia,Drugs, Alcohol, Lead Poisoning, Asplenia can give falsely elevated HbA1C values.
Conditions that shorten RBC survival like acute or chronic blood loss, hemolytic anemia, Hemoglobinopathies, Splenomegaly,Vitamin E ingestion, Pregnancy, End stage Renal disease can cause falsely low HbA1c.

IMMUNOASSAY

TSH (Thyroid Stimulating Hormone) (Serum /Chemiluminescent Immunometric Assay (CLIA))	2.376	µIU/mL	0.35 - 5.50

INTERPRETATION:
Reference range for cord blood - upto 20
1 st trimester: 0.1-2.5
2 nd trimester 0.2-3.0
3 rd trimester : 0.3-3.0
(Indian Thyroid Society Guidelines)
Comment :
1.TSH reference range during pregnancy depends on Iodine intake, TPO status, Serum HCG concentration, race, Ethnicity and BMI.
2.TSH Levels are subject to circadian variation, reaching peak levels between 2-4am and at a minimum between 6-10PM.The variation can be of the order of 50%,hence time of the day has influence on the measured serum TSH concentrations.
3.Values&amplt;0.03 µIU/mL need to be clinically correlated due to presence of rare TSH variant in some individuals.

CLINICAL PATHOLOGY

URINE ROUTINE

PHYSICAL EXAMINATION

Colour (Urine/Physical examination)	Pale yellow	Yellow to Amber
Volume (Urine/Physical examination)	30	ml
Appearance (Urine)	Clear	

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Investigation	Observed Value	Unit	Biological Reference Interval
CHEMICAL EXAMINATION			
pH (Urine)	5.0		4.5 - 8.0
Specific Gravity (Urine/Dip Stick – Reagent strip method)	1.020		1.002 - 1.035
Protein (Urine/Dip Stick – Reagent strip method)	Negative		Negative
Glucose (Urine)	Nil		Nil
Ketone (Urine/Dip Stick – Reagent strip method)	Negative		Nil
Leukocytes (Urine)	Negative	leuco/uL	Negative
Nitrite (Urine/Dip Stick – Reagent strip method)	Nil		Nil
Bilirubin (Urine)	Negative	mg/dL	Negative
Blood (Urine)	Nil		Nil
Urobilinogen (Urine/Dip Stick – Reagent strip method)	Negative		Within normal limits
Urine Microscopy Pictures			
RBCs (Urine/Microscopy)	Nil	/hpf	NIL
Pus Cells (Urine/Microscopy)	1 - 2	/hpf	< 5
Epithelial Cells (Urine/Microscopy)	1 - 2	/hpf	No ranges
Others (Urine)	NIL		Nil

-- End of Report --

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PAP SMEAR

Pap smear by conventional method.

Specimen No:P-217/24
Adequacy: Satisfactory for Evaluation
Transformation zone seen
Predominant cells: Smear studied shows predominantly superficial and intermediate squmous cells. Occasional endocervical cells clusters and metaplastic squmous cells seen.

Background: Moderate neutrophilic inflamatory infiltrate and thin filmy layer of coccobacilli.
Impression: Shift in bacterial vaginal flora suggestive of bacterial vaginosis
Negative for Intraepithelial Lesions/Malignancy

Note:-

PAP smear is a screening Tool. A negative test interpretation does not completely rule out malignancy and should be correlated with clinical findings. Positive findings (if any) are indicative but Not confirmatory of epithelial abnormalities and Need to be further

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evaluated by other diagnostic Methods such as colposcopy, biopsy and Histopathology.

Pap Smear

Pap smear by conventional method.

Specimen No:P-217/24
Adequacy: Satisfactory for Evaluation
Transformation zone seen
Predominant cells: Smear studied shows predominantly superficial and intermediate squmous cells. Occasional endocervical cells clusters and metaplastic squmous cells seen.

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Negative for Intraepithelial Lesions/Malignancy

Note:-

PAP smear is a screening Tool. A negative test interpretation does not completely rule out malignancy and should be correlated with clinical findings. Positive findings (if any) are indicative but Not confirmatory of epithelial abnormalities and Need to be further evaluated by other diagnostic Methods such as colposcopy, biopsy and Histopathology.

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PERIPHERAL SMEAR

RBC's: Normocytic normochromic. Few elliptocytes seen

No RBC inclusions or nucleated RBC seen.

WBC's: Total count is normal in number with normal distribution and morphology.

No immature or atypical WBC's seen.

Platelets: Adequate in number

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Hemoparasites: Not seen.

Impression: Normocytic Normochromic Anemia.

Peripheral Smear Study

RBC's: Normocytic normochromic. Few elliptocytes seen

No RBC inclusions or nucleated RBC seen.

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WBC's: Total count is normal in number with normal distribution and morphology.

No immature or atypical WBC's seen.

Platelets: Adequate in number

Hemoparasites: Not seen.

Impression: Normocytic Normochromic Anemia.

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For BPL

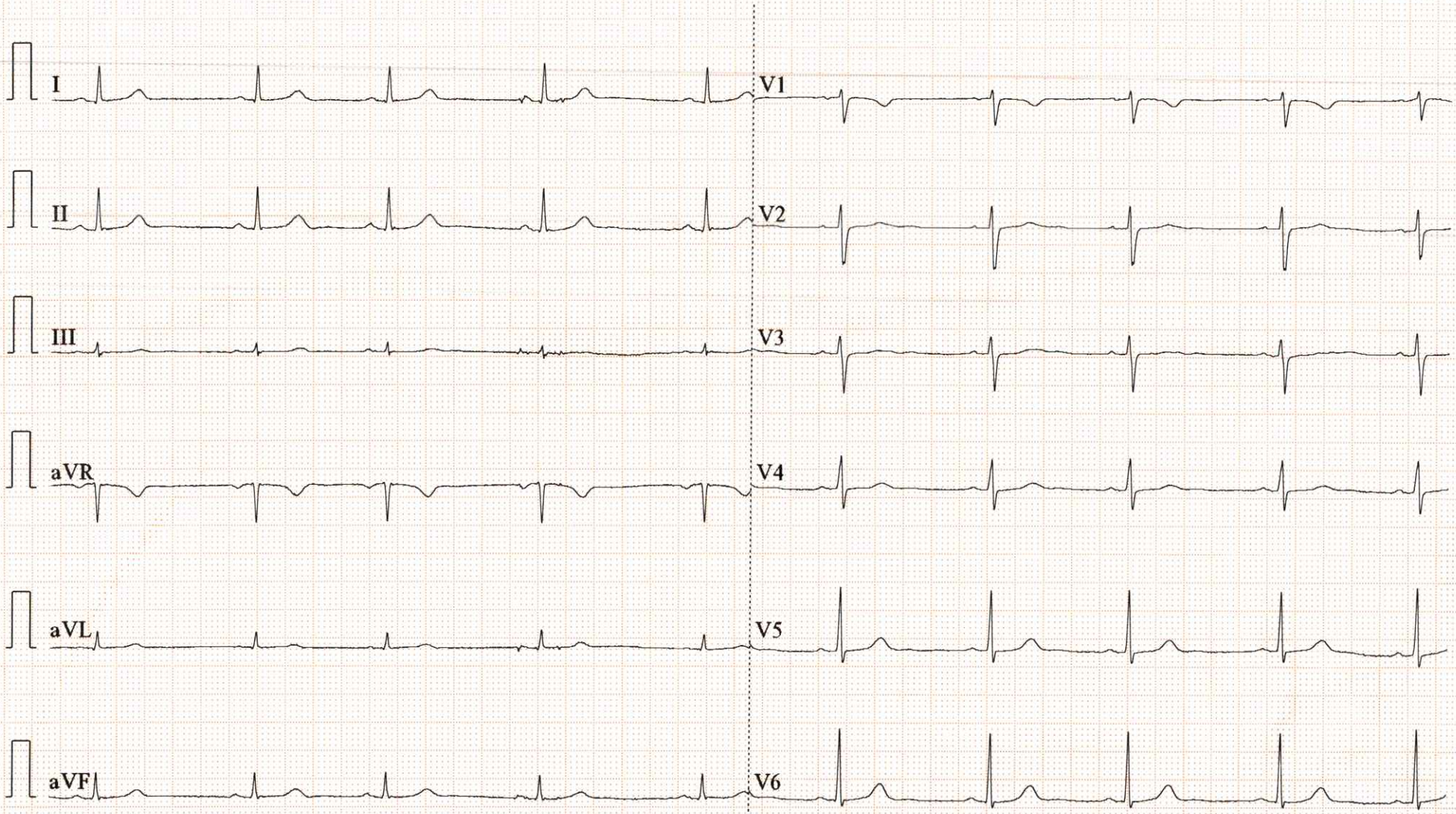
ID: 0
SUPRIYA MAHATME
Female 35Years
Req. No. :

01-06-2024 01:30:44 PM

HR	: 57	bpm
P	: 87	ms
PR	: 139	ms
QRS	: 72	ms
QT/QTcBz	: 425/414	ms
P/QRS/T	: 52/41/44	°
RV5/SV1	: 1.097/0.464	mV

Diagnosis Information:
Sinus Bradycardia with Sinus Arrhythmia

Report Confirmed by:



0.5~45Hz AC50 25mm/s 10mm/mV 2*5.0s V2.21 SEMIP V1.92 DRM DIAGNOSTICS, MYSORE

Trans Thoracic 2D Echocardiography Report

Patient ID: 54682 Date: 01/06/2024

Name: SUPRIYAYA MAHATMA Age: 35yrs Sex: FEMALE

Findings

Left Ventricle	Normal
RWMA	NO RWMA
Left Atrium	Normal
Right Ventricle	Normal
Right Atrium	Normal
IAS	Intact.
IVS	Intact
Aorta	Aortic root, ascending aorta and Arch is normal
Pul Artery	Pulmonary artery is structurally Normal
Valves	Normal
Pericardium	Normal
Doppler	Mild TR

Impression

- **NORMAL CHAMBER DIMENSION**
- **MILD TR / RVSP- 23MMHG**
- **NO REGIONAL WALL MOTION ABNORMALITY**
- **NORMAL LV SYSTOLIC FUNCTION (LVEF - 66%)**
- **LEFT ARCH NO COA**

Dr.MANJUNATH ANVEKAR .

Consultant Cardiologist

Cardiac technologist

DRM MULTISPECIALITY HOSPITAL

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(A Unit of Sri Devi Ventures)

ECHOCARDIOGRAPHY REPORT

Name **SUPRIYA MAHATME** Patient Id **35** Age **35**	Sex **Female** Date **01/06/2024**

M-MODE		MV- DOPPLER		AV - DOPPLER		TV & PV DOPPLER	
IVSd	1.1 cm	MV E Vel	1.02 m/s	AV Vmax	1.02 m/s	PV Vmax	0.95 m/s
LVIDd	3.2 cm	MV DecT	141 ms	AV maxPG	4.19 mmHg	PV maxPG	3.58 mmHg
LVPWd	1.2 cm	MV Dec Slope	7.3 m/s²			TR Vmax	2.41 m/s
IVSs	1.0 cm					TR maxPG	23.28 mmHg
LVIDs	2.0 cm	MV A Vel	0.49 m/s			RAP	5.00 mmHg
LVPWs	1.0 cm	MV E/A Ratio	2.09			RVSP	28.28 mmHg
EDV (Teich)	40 ml						
ESV (Teich)	13 ml						
EF(Teich)	66 %						
%FS	36 %						
SV(Teich)	26 ml						

IMAGE 1

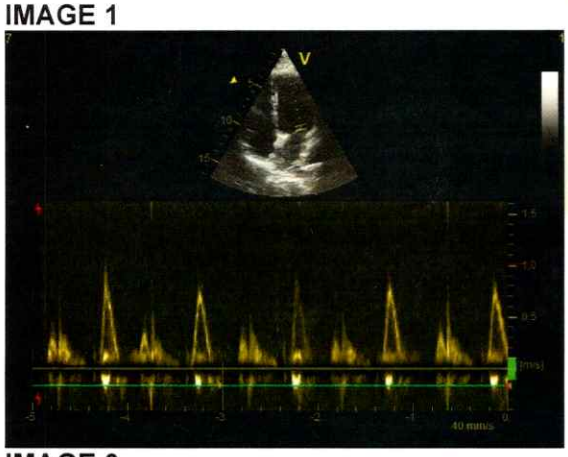


IMAGE 2

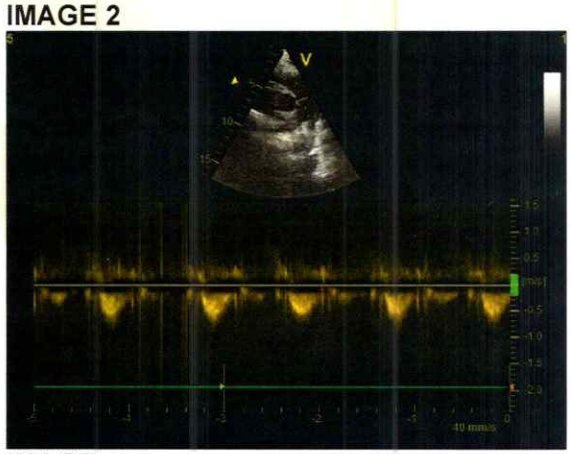


IMAGE 3

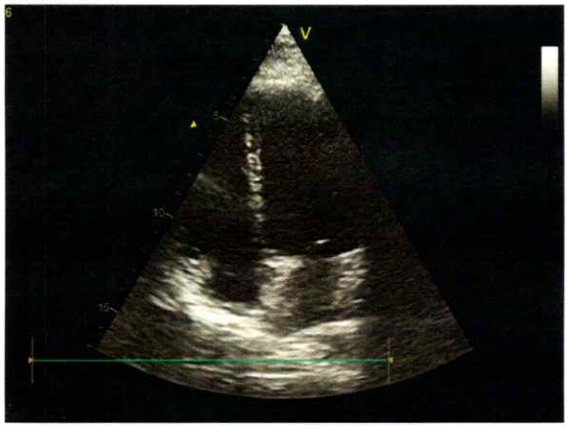


IMAGE 4

2909, Temple Road, Vontikoppal, V.V Mohalla, Mysuru - 570 002

Ph: 0821-2411344 / 2411433 Helpline : 81054 24247

Name	Mrs. Supriya Mahatme	Date	01.06.2024
Age	35 Years / F	MRD NO	L&T

USG WHOLE ABDOMEN

LIVER: Normal in size and echotexture. No focal lesion seen.
Intrahepatic biliary radicals are normal. Portal vein is normal.

GALL BLADDER: Adequately distended. Wall thickness is normal. No calculus / mass lesion is seen. CBD is normal in caliber with clear lumen.

PANCREAS: Normal in size and echotexture.

SPLEEN: Normal in size and echotexture.

KIDNEYS: Both kidneys are normal in size and echotexture. Cortical thickness is normal on both sides. Corticomedullary differentiation is maintained on both sides. No calculus / hydroureteronephrosis seen.

URINARY BLADDER: Well distended. Wall thickness is normal. No calculus / mass lesion is seen.

UTERUS: Normal in size and is retroverted. Echotexture is normal. No mass lesion is seen. Endometrial thickness is 3.5mm.

OVARIES: Normal in size and echotexture. No adnexal solid /cystic mass lesion.

No significant abdominal lymphadenopathy is seen.

No free fluid is seen in abdomen.

IMPRESSION:

- **No significant abnormality detected in abdomen and pelvis.**

DR. LOHITH. K, MBBS, DNB
Consultant Radiologist

DEPARTMENT OF RADIOLOGY

Name	Mrs. Supriya Mahatme	Date	01.06.2024
Age	35 Years / F	MRD NO	L&T

X – RAY CHEST PA VIEW

Cardia is normal in size and of normal configuration.

Both lung fields are clear.

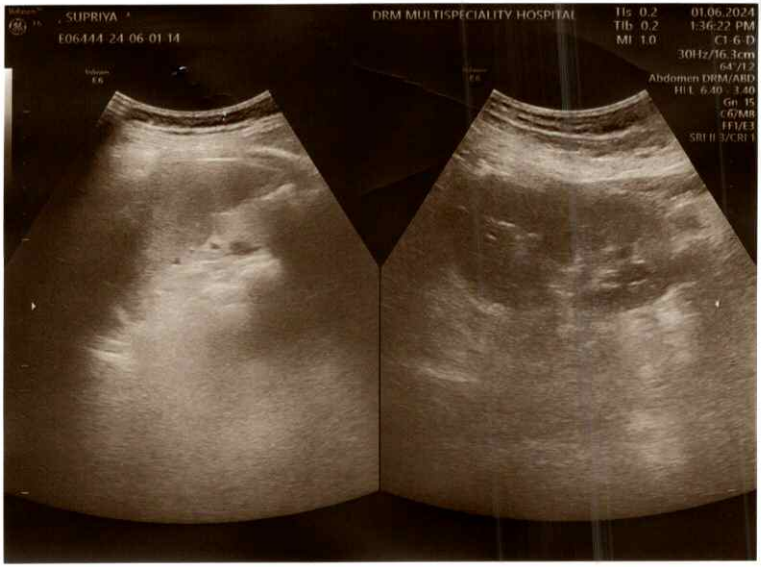
There is no evidence of any parenchyma or pleural pathology.

Both C.P. Angles, domes of diaphragm, Thoracic cage and soft tissue are normal.

IMPRESSION:

- NORMAL CHEST X RAY.

DR. LOHITH. K, MBBS, DNB
Consultant Radiologist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30Hz/16.3cm
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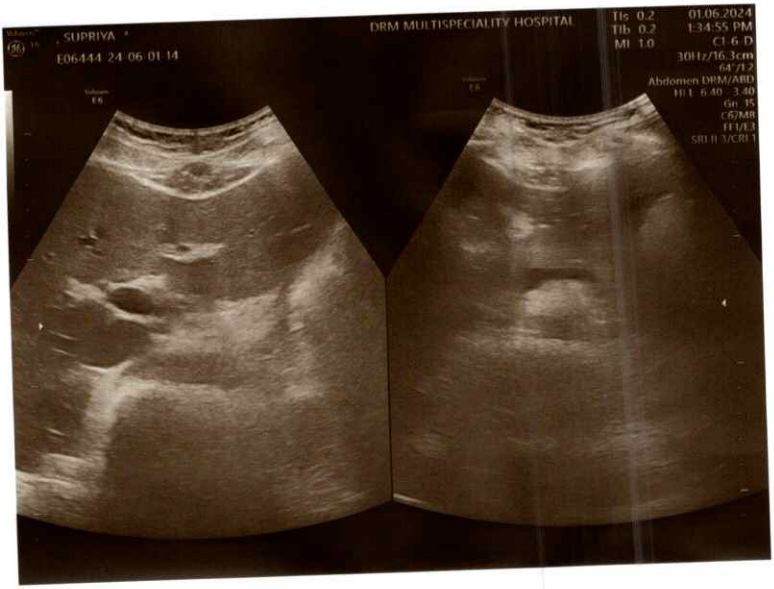
Abdomen DRM/ABD
H/L 6.40 - 3.40
Gn 15
C6/M8
FF1/E3
SRI II 3/CRI 1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C6/M8
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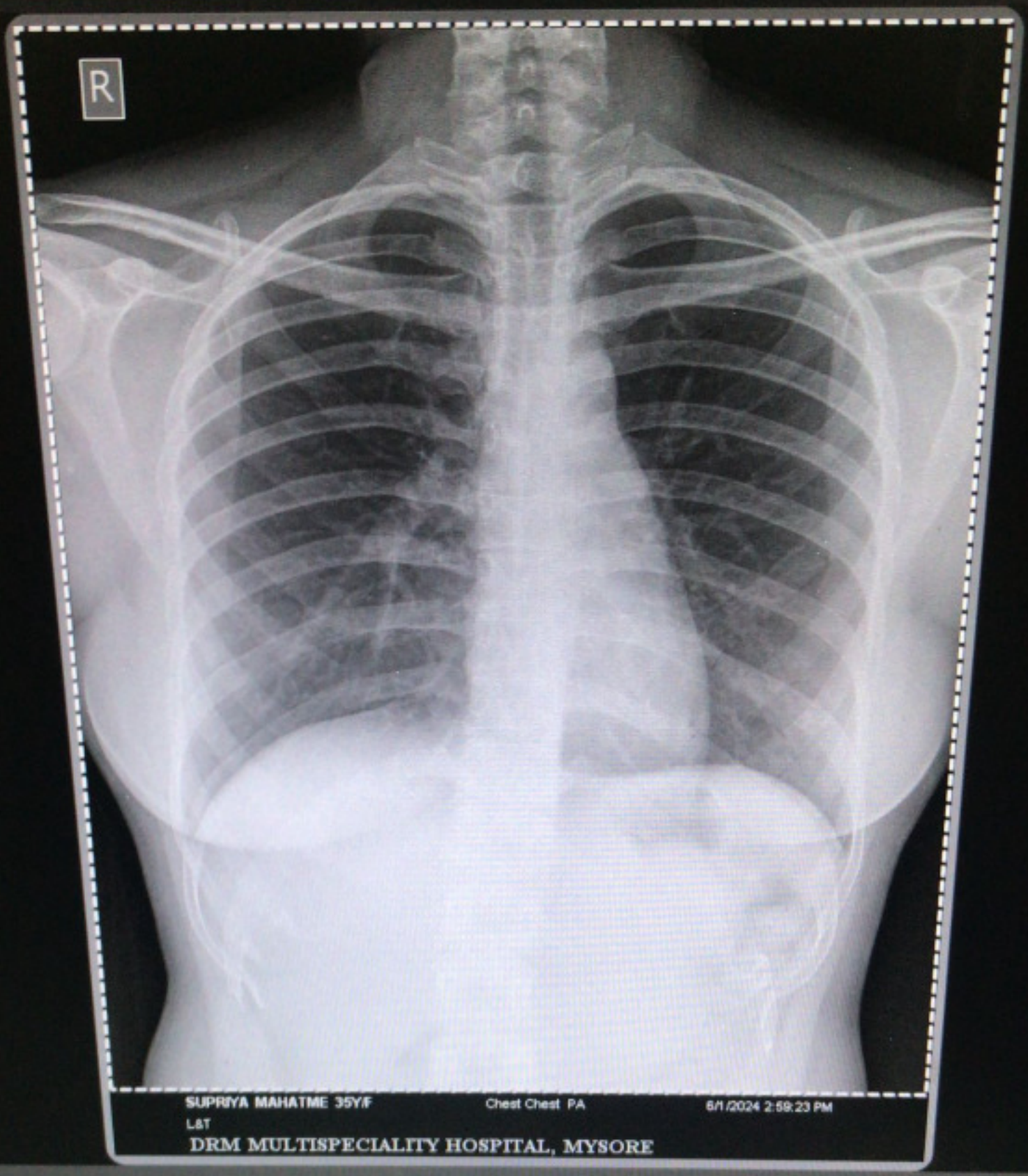
R
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Chest Chest PA
6/1/2024 2:59:23 PM
L&T
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